

Nowhere to Turn

Alberta's Healthcare Worker Shortage and Its Impact on Complex Patients, Disability Documentation, and AISH Recipients

650,000 Albertans without a family doctor. 81% drop in practices accepting new patients. 61% of family physicians considering leaving. And 79,000 AISH recipients required to obtain physician-completed paperwork to keep their benefits.

The Alberta Disability System Breakdown — April 2026 Report Series

The ADAP transition requires all 79,000 AISH recipients to obtain a physician-completed medical report if they want to remain on AISH. Alberta has 650,000 people with no family doctor. 61% of existing family physicians are considering leaving. The very transition that demands physician paperwork is happening simultaneously with the collapse of the primary care system that produces it.

EXECUTIVE SUMMARY

Alberta has a healthcare worker shortage that is severe, documented, and worsening. More than 650,000 Albertans — more than the entire population of Calgary — have no family doctor. The number of practices accepting new patients has fallen from 887 in 2020 to 164 in 2024. Over 60% of family physicians are considering leaving the provincial healthcare system. And the patients being hit hardest are not the healthiest ones.

For Albertans with disabilities on AISH — people whose qualifying conditions are complex, chronic, and require coordinated multi-specialist care — the healthcare worker shortage is not an inconvenience. It is a structural threat to their ability to manage their health, document their disability, access benefits, and survive. Every mandatory form that requires a physician signature, every specialist referral that requires a GP, every prescription renewal that requires a visit — all of these become crises when there is no GP.

SECTION 1: THE SCALE OF ALBERTA'S PRIMARY CARE COLLAPSE

1.1 The Numbers

METRIC	FIGURE	SOURCE
Albertans without a family doctor	650,000+	Alberta Medical Association (AMA)

METRIC	FIGURE	SOURCE
Family practices accepting new patients — 2020	887	Alberta Find a Doctor / U of Calgary, 2025
Family practices accepting new patients — 2024	164	Alberta Find a Doctor / U of Calgary, 2025
Decline in accepting practices	81% reduction in 4 years	Calculated from above
Family physicians considering leaving provincial system	61%	ThinkHQ survey, AMA, January 2024
Family physicians considering leaving healthcare entirely	Over 25%	ThinkHQ survey, AMA, January 2024
Canadians nationally without a family doctor	~8.5 million (15%)	Canadian Medical Association Journal, 2024
Additional family physicians needed to fill Alberta's gap	Estimated 678–715 FPs	Canadian Family Physician journal, 2025
Additional nurse practitioners needed (alternative)	Estimated 722 NPs	Canadian Family Physician journal, 2025

Sources: Alberta Medical Association — albertadoctors.org; University of Calgary news release, September 23, 2025; ThinkHQ/AMA survey, January 2024; Canadian Family Physician journal, 2025.

“People are older and sicker. The number of people with five or more health conditions — like a history of cancer, high blood pressure and heart issues — nearly doubled.” — Dr. Braden Manns, University of Calgary, lead investigator, September 2025

“Alberta's family medicine system is collapsing. Across the province, upwards of 650,000 Albertans are searching for a family doctor. Without a family doctor, chronic conditions are almost impossible to manage and new issues, including cancers, may go unnoticed until they are past the point of effective treatment.” — Alberta Medical Association, official statement

1.2 Why It's Getting Worse, Not Better

The paradox at the centre of Alberta's family doctor crisis is that the number of registered family physicians is increasing — and yet access is collapsing. In the first quarter of 2024, there were 4,374 family physicians registered in Alberta, 215 more than the same period the previous year. But they are not taking patients.

- Physicians doing comprehensive, longitudinal care earn less per hour than those doing walk-in or urgent care under the previous fee-for-service model. Complex patients take longer. The billing model does not reflect the workload.
- Alberta's new Primary Care Physician Compensation Model (PCPCM), introduced in 2025, attempts to address this — but critics warn it introduces a two-tier system where complex patients on fixed incomes fall further behind.
- Burnout among family physicians is documented and severe. Alberta's College of Family Physicians president stated: *"Everything we've heard and seen is that family physicians and the primary care system are breaking down."*
- Rural Alberta has been hit disproportionately. In some regions, the nearest GP accepting new patients is a 90-minute drive away. For AISH recipients without vehicles, rural GP access is effectively zero.
- Family medicine residency vacancies are climbing nationally: in 2023, there were 268 unfilled family medicine residencies across Canada. The pipeline of new family physicians is contracting at exactly the moment demand is expanding.

1.3 The Two-Tier Risk

Alberta is the only province implementing a hybrid public-private physician compensation model. Under this model, physicians can choose to bill the government or bill patients privately. Critics warn that the result will be a two-tiered system where those who can pay privately access care faster, and those who cannot — including 79,000 AISH recipients — fall to the back of every queue.

*For AISH recipients on fixed incomes, a two-tiered healthcare system is not a theoretical concern. It is a direct and immediate threat to continuity of care. **They cannot pay privately. They cannot jump queues. They are, by definition, the patients most likely to be left at the bottom of every tiered system the market creates.***

SECTION 2: WHAT THIS MEANS FOR COMPLEX PATIENTS

In the context of Alberta's AISH population, "complex patients" describes people with multiple co-occurring permanent conditions — immune disorders, psychiatric conditions, chromosomal syndromes, neurological disorders, degenerative disease — who require coordinated care across multiple specialists, whose conditions interact with each other and with their medications, and whose health cannot be managed through episodic walk-in visits.

2.1 Why Complex Patients Are Being Left Behind

- Complex patients take more time per appointment. Under fee-for-service billing, a complex patient with 8 conditions, 12 medications, and 4 specialists generates more than 4 times the work of a simple patient — and the same or less billing.

- Complex patients require more coordination: referrals, letters, forms, specialist communications, prescription renewals, school and benefit documentation. This administrative load falls on the GP — often uncompensated under standard billing codes.
- Complex patients are less likely to be accepted when a practice has room for new patients. A physician managing a waiting list will rationally accept patients whose needs are lighter.
- The collapse of longitudinal care is most harmful for complex patients. A walk-in physician who sees someone once cannot safely manage a patient on 12 medications with interacting conditions.
- Without a GP, there is no referral pathway. Without a referral, there is no specialist. For conditions that require specialist management, no GP means no care.

“With quick and regular access to care, health problems can be caught and treated earlier. Whereas the longer you wait, the worse it gets. And so people present with delayed pathology.” — Dr. Everett Zdrill, family physician, Edson, CBC News

2.2 The AISH Population Is Disproportionately Complex

The AISH caseload, by its nature, represents Alberta’s most medically complex residents. The qualifying criteria require a severe, permanent disability that substantially prevents earning a livelihood.

AISH PRIMARY CONDITION	% OF CASELOAD	WHY GP ACCESS MATTERS
Physical disability	41.5%	Requires ongoing medication management, specialist coordination, and regular monitoring
Mental illness disorder	30.2%	Requires psychiatric medication oversight, crisis management coordination, specialist referrals
Cognitive disorder	28.3%	Requires developmental coordination, behavioral management, and documentation for disability services
All categories	100%	All AISH recipients require physician-completed medical documentation for program access and renewal

Source: Alberta Assisted Living and Social Services Open Data — AISH Caseload September 2025

SECTION 3: THE SPECIFIC BARRIERS FOR AISH RECIPIENTS

3.1 No GP Means No Referral — And No Referral Means No Specialist

In Alberta's healthcare system, most specialist care requires a GP referral. Without a GP:

- A person with a psychiatric condition cannot be referred to a psychiatrist — they must present to an emergency department or a walk-in clinic and hope for referral.
- A person with an immune deficiency cannot be referred to an immunologist for treatment changes — leaving them managing a life-threatening condition without specialist oversight.
- A child with a cardiac condition and a rare chromosomal syndrome cannot be referred to a developmental pediatrician without a GP initiating that process.
- Prescription renewals for controlled medications or specialty drugs may be refused by walk-in physicians unfamiliar with the patient's complex history.

THE EMERGENCY ROOM AS DEFAULT: *When people with complex needs have no GP, emergency rooms become their primary care. This is expensive for the system, harmful for the patient, and creates the exact kind of fragmented, episodic care that is most dangerous for people with multiple interacting conditions. The healthcare system saves money by not funding comprehensive family medicine and spends far more in emergency and acute care on the same patients.*

3.2 No GP Means Forms Cannot Be Completed

AISH, ADAP, and DTC applications all require physician-completed forms. The healthcare worker shortage directly intersects with the administrative burden these programs place on recipients:

- With only 164 practices accepting new patients province-wide, an AISH recipient without a GP cannot access a physician to complete their AISH application, their ADAP reassessment form, or their DTC application.
- Walk-in physicians will frequently decline to complete complex disability forms for patients they have seen once — they do not know the patient, cannot accurately represent a complex disability history, and completing the form exposes them to liability.
- Nurse practitioners — who often do know complex patients — still cannot sign all sections of AISH forms under current rules, despite being registered, regulated professionals with equivalent knowledge of their patients.
- The ADAP transition's requirement that 79,000 recipients obtain a physician-completed Disability Assistance Medical Report is happening simultaneously with the worst primary care access crisis in Alberta's recorded history.

3.3 Rural Recipients: The Double Barrier

For AISH recipients in rural and small-town Alberta, the healthcare worker shortage is not a theoretical future concern — it is the current daily reality.

- In the Bow Valley region, the nearest GP accepting new patients was a 90-minute drive away as of early reporting. For an AISH recipient without a vehicle and with mobility limitations, a 90-minute drive may as well be a flight.
- Since 2020, six family physicians left the Bow Valley primary care network alone. The loss of even one or two family physicians can strip an entire community of primary care access.
- One documented patient moved from their rural community to Lethbridge specifically because of the doctor shortage — then had to drive back to their original community for specialist care. For AISH recipients who cannot drive, moving to a city is not a solution: it simply relocates the crisis.
- Telehealth options are insufficient for patients requiring physical examination, blood draws, wound care, immunoglobulin infusions, or equipment management — all common requirements in the AISH population.

“Not knowing is difficult. Consistent care is key for maintaining my health. It just leaves me kind of not knowing how will my health be cared for.” — Daphne Pauls, 53, fibromyalgia patient whose family doctor left the province, CBC News

3.4 The Children: Complex Care Plans in a Collapsing System

For AISH recipients who are parents of children with complex disabilities — rare genetic syndromes, autism, cardiac conditions, intellectual disabilities — the healthcare worker shortage affects not only their own health management but their children's ongoing medical care.

- Developmental pediatrician oversight requires a GP referral to access — a specialty with long wait times even when the referral pathway exists.
- Coordinated care plans involving multiple specialists, schools, and community agencies fall apart without a consistent GP holding the centre.
- Emergency protocols for conditions like cardiac monitoring requirements cannot be established without a physician who knows the child's full history.
- Annual renewals of FSCD and other disability service contracts require updated medical documentation that must come from a physician.

*A single parent on AISH with multiple children who have complex disabilities and medical needs requires coordinated GP-initiated referrals to developmental pediatrics, cardiology, psychiatry, and school-based health services — simultaneously. This coordination requires a GP who knows the whole family. **In a system where 650,000 people have no GP, this coordination often does not happen.** The consequences are missed diagnoses, delayed interventions, and health crises that could have been prevented.*

3.5 The Medical File Problem: Physician Retirement and Missing Records

The healthcare worker shortage produces a secondary crisis that has received no formal policy attention: when a family physician retires or leaves a practice, their patients do not automatically receive their medical files, and file transfer to a new physician is not guaranteed. For AISH recipients with complex, longstanding medical histories, this creates a documented and specific threat to ADAP reassessment accuracy.

- Medical files are the property of the physician or clinic, not the patient. When a physician retires, the obligation to transfer files is not consistently enforced.
- AISH recipients whose GPs retire often attempt to obtain their complete medical files in advance of the retirement — contacting both the CPSA and the College of Family Physicians to understand the process and associated fees.
- The CPSA confirms that physician billing questions should be directed to the Alberta Medical Association — not the CPSA. No clear regulatory pathway for file access fees is identified.
- When a physician retires and no formal file transfer occurs, the patient's new physician — when one is eventually found in a system with 164 accepting practices — does not have the complete medical file.
- That recipient now faces an ADAP medical reassessment. The physician completing that reassessment does not have access to the clinical history required to complete it accurately.
- For a recipient whose qualifying condition is complex and longstanding, a physician completing the ADAP Disability Assistance Medical Report without the full file may be unable to accurately document the severity, duration, and functional impact of the disability — the three factors on which program eligibility is determined.

RISK FACTOR	IMPACT ON ADAP REASSESSMENT
Physician retirement without complete file transfer	New GP cannot accurately document disability history, severity, or functional impact
New GP with no prior patient relationship	Walk-in physicians may decline to complete complex disability assessments for new patients
Incomplete clinical record	ADAP medical panel receives incomplete information — eligibility determination may not reflect actual condition
No regulatory enforcement of file transfer	Patient has limited recourse if file is not transferred before physician retires
Appeal rights removed by Bill 12	If ADAP panel makes incorrect determination due to incomplete file, there is no independent appeal

Sources: CPSA — cpsa.ca; Alberta Medical Association — *billing and uninsured services guidance*.

*The combination of factors is compounding: a physician retires, a file is not transferred, a new GP is difficult to find, the new GP lacks clinical history, the ADAP reassessment form cannot be completed accurately, and the decision cannot be appealed. **Each of these is a documented systemic failure. Together they form a pipeline that leads from GP retirement to incorrect program termination — with no independent mechanism to catch or correct the error.***

SECTION 4: THE NURSE PRACTITIONER BARRIER

Nurse practitioners (NPs) are regulated, highly trained healthcare professionals who in many cases have longer and more established relationships with complex patients than the physicians required to sign their forms.

“Why would I have to get a doctor to sign a form for my own patient? So, that’s ridiculous. Number 2: there aren’t enough family physicians for people to go to. I have a number of adult FASD patients now that we’re getting assessed, and they should qualify for AISH, but I can’t sign the medical.” — Nurse practitioner Wonitoway-Raw, Edmonton Journal, January 11, 2024

This is not an isolated frustration. The government has announced expanded scope of practice for nurse practitioners to address the family doctor shortage, while simultaneously maintaining rules that require physician signatures on disability benefit forms — forms that NPs are professionally and clinically qualified to complete.

FORM TYPE	NP CAN CURRENTLY SIGN?	WHY THIS MATTERS
AISH Part B Medical Report	Partially — physician signature still required for key components	NPs may know the patient better than any GP. The restriction creates a barrier with no clinical justification.
DTC Form T2201	Yes — NPs are now included	A positive change, but awareness is low among both NPs and patients.
ADAP Disability Assistance Medical Report	Rules still being established	Critical — if NPs cannot sign this form, GP-less Albertans face an impossible barrier.
Specialist referrals	Varies — NPs can refer in many contexts but not all	Specialist access depends on the referral pathway being fully open to NPs.

Sources: Edmonton Journal, January 11, 2024; Alberta.ca — AISH application guidelines; ADAP Discussion Guide, August 2025.

SECTION 5: WHAT IS BEING ASKED

To the Government of Alberta

- Immediately expand the list of professionals authorized to complete all AISH, ADAP, and DTC-related medical forms to include nurse practitioners, registered psychologists, and specialists with established treating relationships — with no physician countersignature required.
- Invest urgently in primary care capacity — specifically in longitudinal, comprehensive family medicine for complex patients. The current compensation model must be corrected before the PCPCM hybrid model accelerates the two-tier risk.
- Fund and mandate complex care coordination teams specifically for high-complexity patient panels including AISH recipients.
- Establish a provincial guarantee of primary care access for AISH recipients. A person who qualifies for AISH on the basis of a permanent, complex disability should not be left without a GP.
- Establish a mandatory medical file transfer protocol when a physician retires or closes a practice, with patient notification and a guaranteed timeline. For AISH recipients facing ADAP reassessment, an incomplete medical file is not an administrative inconvenience — it is a threat to program eligibility.
- Pause the requirement for physician-completed ADAP medical reports until primary care access has been meaningfully improved. Requiring 79,000 people to obtain a form signed by a physician, in a province where 650,000 people have no physician, is not a workable policy.

To the Government of Canada

- Permanently fund the completion of DTC medical forms as a covered service — not as a one-time supplemental payment. The \$150 one-time supplemental payment does not cover current physician fees and does not address the ongoing barrier.
- Formally recognize nurse practitioners as eligible signatories for all federal disability benefit forms — including the DTC T2201 — and communicate this clearly to NPs and patients across Canada.
- Include access to primary care for people with disabilities as a condition of federal health transfer payments to provinces.

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